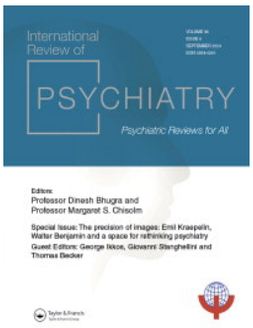

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Emil Kraepelin and Walter Benjamin: distant contemporaries, diverse working methods, any resonance?

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ABSTRACT

The work methods of eminent psychiatrist Emil Kraepelin (1856–1926) and of literary and cultural critic Walter Benjamin (1892–1940) are described based on the literature describing their ways of working. Kraepelin's approach of cross-sectional observation of symptoms and the longitudinal study of illness course in distinguishing 'disease entities' as well as his research strategy of investigating causes and correlates using methods of experimental psychology, pathology and neurobiology are described. Critical statements (on Kraepelin's work) highlighting preconceived concepts, ideological positions and a lack of critical reflection are reported. Walter Benjamin's use of citation and 'collage' in literary and cultural criticism, his use of the concepts of allegory, 'figures of thought' and 'dialectical images' is described. His theory of the past impacting on our understanding of present and immediate future is discussed. Benjamin's thinking is centered around the concept of 'experience'. The relevance of both scientists to current challenges is discussed referring to current psychiatric research and peer support, a key concept in discussions on mental health care. The paper argues that Kraepelin and Benjamin could be considered complementary in their approaches to mental health matters taking into account wider cultural contexts. (189 words)

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Emil Kraepelin; Walter Benjamin; psychiatry; mental health research; peer support

Introduction

Emil Kraepelin (1856–1926) and Walter Benjamin (1892–1940) shared about a decade and a half of their adult lives. They did not share a profession, they did not refer to each other in their writings, and their views on history, society and politics were wide apart, with Kraepelin being close to a position we could describe as nationalistic (with Chauvinism in it, or 'deutschnational') while Benjamin had a marked interest in Karl Marx 'writings and was clearly left-leaning.

Emil Kraepelin was the most influential German psychiatric scientist, practitioner and academic of the late 19th and early 20th century. His work has been a key reference point in psychiatry in the one hundred years following his death, and it has had a lasting influence on psychiatric nosology and science. Walter Benjamin is considered one of the foremost literary critics, cultural scientists, translators (and unorthodox Marxist thinkers) of the German language of the first half of the 20th century. His work has been widely referred to since Benjamin's death in 1940. In post-1945 literary and cultural science (and in the humanities at large) Benjamin's work has been among the most influential in Western thought in the second half of the 20th century. The thinking of both these scientists had philosophical underpinnings, with Kraepelin's thinking being founded in philosophical realism and, as for the mind-body-problem, in parallelism (Hoff, 1992), and Benjamin, in terms of philosophy, referring mostly to Plato, Leibniz and Kant.

This text attempts, in part 1 (on Kraepelin) and in part 2 (on Benjamin) to describe their respective theoretical considerations, tenets and working methodologies. Looking at Kraepelin and Benjamin in one paper is justified by prior publications suggesting that Walter Benjamin's thinking may be relevant to psychiatry with a view to psychiatric theory, practice and professionalism (Ikkos & Stanghellini, in press; Stanghellini, this volume; Ikkos, 2024; Stanghellini &

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Ikkos, 2024). Stanghellini and Ikkos (2024), in a paper on Charles Baudelaire's poetry, argue that its interpretation in Walter Benjamin's texts may help adding depth to psychiatric clinical practice in dealing with depression. The authors have also explored contributions that Benjamin made in other fields of psychiatric theory and practice and the field of psychiatric professionalism (Ikkos, 2024). On this basis, the intention of the present paper is to explore whether aspects of Benjamin's thinking may complement Kraepelin's (and other psychiatric authors') understanding of mental ill health and provide a form of language for mental (ill) health and psychiatric issues extending beyond psychiatric science. Parts 1 and 2 are discussed separately, and the paper is concluded by a general discussion that aims to pull together and integrate both parts.

Part 1: Emil Kraepelin's working methods and research strategy

Materials and methods

This section aims to describe and discuss Emil Kraepelin's clinical and research methodology and practice. On the basis of a non-systematic informed literature search, it draws on publications in psychiatry and history of medicine scientific journals and books, among which Hoff (Hoff, 1994) provides a comprehensive account.

Results

Influences on Kraepelin: Kahlbaum and Hecker

Kendler and Engstrom (2017) discuss the influence of the work of Karl Kahlbaum and Ewald Hecker on Emil Kraepelin's approach to psychiatric nosology. Both Kahlbaum and Hecker worked at the Görlitz mental health institution in the second half of the 19th century. The paradigm shift brought about by Kahlbaum and Hecker (based on work from about 1860–1880) moved from symptom-based syndromes to proto-disease entities focusing on clinical course. Their thinking was influenced by the rise of bacterial theories of disease and by progress in understanding 'general paralysis of the insane' (GPI). Kahlbaum and Hecker contributed to our understanding of catatonia and hebephrenia, and they believed in a clinical resarch agenda utilising both clinical assessment and follow-up in defining forms of disease. They criticised the older 'neuropathological paradigm' that was dominant at the time.

Counting cards: instrument of clinical research

Weber and Engstrom (1997) deal with the working method that Kraepelin used in developing the concept of endogenous dementias (endogene Verblödungen) and the dichotomy of dementia praecox vs manic-depressive illness. The paper looks into the assumptions behind the so-called counting cards (Zählkarten) which Kraepelin used to organise his clinical observations. It inquires into the historical origins, textual structure and content of the cards. The authors trace the origins of the counting cards in medical and population statistics which, in turn, had their origin in what the authors call the political arithmetic of the Enlightenment. In 19th century Europe, psychiatric statistics were products of administrative practice and health policy. Weber and Engstrom provide impressive evidence of institutional mental health statistics published in association with the establishment of mental health institutions across Europe. In 1871, a census of the 'insane' was performed in the state of Prussia using a scheme of 11 tables. In the attempt to improve census quality, a staff member of the Prussian statistical bureau proposed a 'Zählkarte für Geisteskranke' (census card for mentally ill) comprising 12 items (personal, hereditary and psychopathological data). The model of the so-called 'Individualzählkarte' (individual census card) of the population census was used. The authors report that there was a similar practice of collecting statistical information in Bavaria where Kraepelin worked early in his career. In his later work, Kraepelin described the counting card system as a new and longitudinal method of clinical observation. The aim was, as Kraepelin argued, to separate essential from incidental features of syndromes. Kraepelin attempted to follow up patients longitudinally (with that information added to the counting cards) but reported that this was not always possible (due to time constraints). Kraepelin stated that the mass of information contained in patients' clinical files had to be condensed to what he considered the 'important' facts. Thus, there was an interpretative process at work in condensing patient reports to the format of the counting cards, and this implied cognitive discrimination and scientific assessment based on preconceived categories.

On the basis of 705 counting cards in the archive of the Max-Planck-Institute of Psychiatry in Munich Weber and Engstrom (1997) report that the counting cards included about 30 items of personal and illness-related data (aetiology, heredity, medical history, age at first and current onset, duration of treatment, psychopathological status, course of symptoms, diagnoses, and additional data, e.g. forensic issues or

anatomical data). The cards did not contain any information on biography or psychological development. The majority of patients with counting cards were women of lower social classes. About 46% of the cards contained no diagnostic categories, the other half included a mix of different diagnoses with dementia praecox amounting to only 4% of the cards, catatonia was recorded in 3%, hebephrenia in 1% and manic-depressive disorder in 5% of cases. More than 50% of the counting cards examined by the authors did not provide any information on illness course, and in about half there was no entry in the categories of 'heredity' or 'etiology'. The description of psychopathology was dominated by generic descriptive terms, with psychomotor symptoms described more frequently than other symptoms. The authors take this as indicating 'imprecision and lack of terminologial rigor' (Weber & Engstrom, 1997, p 382).

Weber and Engstrom (1997) conclude that Kraepelin's counting cards give the impression of his having collected empirical information which helped to strengthen 'preconceived concepts' and that his nosological concept should be seen as a 'theoretical ideal' not substantially verified by clinical records. The counting card system fit the positivist research agenda that Kraepelin had initiated, and it also served taxonomic, didactic and public health purposes. In an editor's comment (published along with the paper) German Berrios points to the fact that Kraepelin (as other renowned scientists) was guided by a priori views as to how objects in the field of inquiry should be defined and ordered and that the counting cards could not be of scientific use for outcome studies.

The role of laboratory science: premises and implications

Volker Roelcke (1999b), in a book chapter, argues that Kraepelin's clinical research placed a selective focus on certain phenomena and patient data while ignoring or not referring to others. He also argues that the psychiatric nosology that Kraepelin developed reflects his selective research strategy. Roelcke refers to the importance of Kraepelin in current classifications of psychiatric diseases (Neo-Kraepelinian turn). He argues that Kraepelin's research was influenced by his early laboratory studies in experimental psychology (of the early 1880s). There was a focus on causality (necessary causes) and on quantifying elementary mental processes as a foundation of psychiatric nosology. In his dissertation 'Über den Einfluß akuter Krankheiten auf die Entstehung von Geisteskrankheiten,' Kraepelin had attempted to break down clinical phenomena into elementary mental processes and, within these processes, to analyse symptoms with a view to causal factors. He worked in the laboratory led by Wilhelm Wundt (Araujo, 2016). Empirical data were to be used in understanding and classifying mental disorders. Roelcke argues that Kraepelin did not reflect or discuss the presuppositions of his research paradigm. He considers the foundation of mental disorders on a somatic basis the most important accomplishment of 19th century psychiatry. Kraepelin used delirious states in febrile conditions as the object of study to establish causes. He started by using the concept of predisposing factors but aimed to break down the phenomena into 'psychological elementary processes' (supposedly related to neurophysiological facts). Kraepelin looked at cases of febrile delirious conditions in the literature and concluded that delirious symptoms disappeared when the febrile condition resolved. He thought that elevated body temperature acted upon the central nervous system. In dealing with this example Kraepelin coined the term of 'significant cause' (meant to signify a causal factor that is mandatory for the mental phenomenon to arise). Roelcke (1999b) considers Kraapelin's approach as the 'mechanics of nerves' where Kraepelin thought that isolating elementary causal relations would be more productive than using static morphological models. Experimental psychology appeared the most promising avenue to understanding mental disorders. Kraepelin held that 'psychophysical parallelism' was the philosophical foundation of his work. His optimism was related to Wundt's research into 'perceptual processes'. Kraepelin contributed latoratory results on stimulus-reaction times leaving all conditions constant with the exception of variable drug applications. He used the study of alcohol effects on 'choice' and 'discrimination' as one example. There was another predetermined element in Kraepelin's attempt to build a classification system, i.e. he postulated disease entities that would help identify specific causes. Roelcke describes a 'heuristic' and a 'normative' aspect in Kraepelin's experimental model.

According to Kraepelin, establishing diagnosis was possible, using one of the following three pathways: (1) identifying the causal factor (or causes), (2) establishing pathological findings, or (3) analysing the clinical picture and its course. Kraepelin felt that diagnostic clarity would also contribute to the prevention of mental disorders. As there were clear methodological limitations in using pathways (1) and (2) it was not surprising that Kraepelin concentrated on the description of clinical phenomena both cross-sectionally and longitudinally. He used the counting cards to

systematise clinical description and documentation. There was a focus on heredity and other causes. Physical state, neurological findings, psychomotor and cognitive performance (but not biography or family relations) were documented. Kraepelin thought that this avenue of research would provide psychiatry its position as a truly medical discipline. He sought to achieve this aim on the basis of a research programme driven by pre-empirical premises (and believing in mechanisms similar to those identified in communicable disease). In a handbook chapter, Roelcke ((Roelcke, 1996) states that Kraepelin was convinced that the juncture between physical and mental processes could be analysed by using experimental psychology. According to Roelcke it was the experimental model that determined the ideas of specific causation (one cause, one disease) and the conviction that there were distinct disease entities. There was a strong focus on biological constructs against the background of Kraepelin's 'psychophysical parallelism'.

Mental health statistics: research tool and public health instrument

Roelcke (2003) describes how, around the 19th to 20th century turn, Kraepelin's research strategy and his nosological system contributed to increasing the academic and institutional power of psychiatry, and how the use of methods from the psychology laboratory shaped psychiatric science. He refers to Kraepelin's use of counting cards. Kraepelin was a scientist offering solutions to important issues such as mental health statistics (Irrenstatistik) and problems of terminology and classification.

In the field of public mental health statistics, arriving at reliable and valid data on mental illness prevalence was important to improve the reputation of psychiatry. This, of course, highlighted the need for a classification system of mental disorders based on consensus. A first such survey (for the city of Berlin) was conducted in 1868. There was an initiative to extend this survey across the whole of Prussia. There were terminological controversies (in conferences and professional organisations) around the survey project. Several psychiatrists raised the issue of terminological confusion, and this continued throughout the 1870s. Practitioners criticised the lack of terminological and nosological clarity. Kraepelin's attempt to identify 'elementary processes' related with measurable physiological facts (reaction time, discrimination experiments) was considered promising as was his search of 'specific causality'.

Kraepelin developed his nosological classification based on experience and laboratory research. Neuropathology, neurophysiology, physiological anthropology plus observational and measurement techniques from somatic medicine (e.g. ophthalmoscopy) were mentioned and considered relevant. Kraepelin gave substantial space to etiology in his textbook. The triad of disease entities, specific causes (etiology, combinations of causes) and specific pathological anatomy dominated Kraepelin's thinking. Roelcke suggests that Kraepelin's nosological system was attractive not through new discoveries but because of its coherence and plausibility. It set the stage for a psychiatry dominated by natural science methods. The focus on symptoms was selective favouring behaviour over subjective phenomena.

Building a theory

Roelcke (2002) suggests that the 'biologicisation of man' that prevailed throughout medicine around the year 1900 can help understand the development of German psychiatry during the late Kaiserreich and Weimar Republic. The rise of the paradigm of heredity is considered an example of how the production of scientific concepts, theories and interpretational models had an impact on politicians, public administrators and the public. The paper refers extensively to Alfred Hoche's 'constructivist' critique of Kraepelin's basic tenets and Karl Bonhoeffer's 'exogenous reaction types' as alternative concepts. Roelcke also refers to Eugen Bleuler who concentrated on the content of mental illness symptoms. He took issue with Kraepelin's tenet of deterioration into dementia as a rule (among people suffering from dementia praecox or schizophrenia). Jaspers developed the concepts and methods to be put to use in a descriptive science of the manifestations of mental ill health. Oswald Bumke went on to dismantle dementia praecox as a disease entity and suggested the term of 'schizophrenic reaction types' with diverse lesions or dysfunctional brain states being causal. Hans-Walter Gruhle analysed diagnoses (at the Heidelberg University Clinic) over almost 40 years (1892–1930) and reported on 'waves' (or 'fashions') of diagnostic constructs pointing to the theory- and context-dependence of scientific terms and research programmes, and he asked for a continuous reflection of the premises of scientific concepts. He thought a specific susceptibility (to schizophrenia) was likely and somatic correlates were plausible, but not one specific cause. He ended with impressive modesty regarding the limited knowledge on the matter. Roelcke's text then concentrates on the increasing

public importance (both in the scientific and political arena) of the eugenic paradigm in the course of the second half of the 1920s – with significant contributions by Ernst Rüdin, in particular.

Quantifying, classifying, and counting: norm-setting and methodological issues

Roelcke (1999a) refers to work on 'norm-setting and normalisation' in the sciences by Georges Canguilhem (1974) and Michel Foucault (1975). Foucault assumes that power relations concerning the 'soul' provide power over the 'body'. Roelcke states that, in the historiography of psychiatry, the period 1880–1914 is generally considered a period of 'consolidation'. Kraepelin was key to the building of theory and professionalism in psychiatry. Roelcke describes three methodological perspectives in Kraepelin's research: quantification, classification, and epidemiology. Each of the three has an important contribution to make in paving the way towards a natural science-dominated medicine in the field of mental illness.

Kraepelin aimed at a quantitative empirical research paradigm. He held that by using this paradigm diagnostic assessments, health policies and medico-legal statements would become more stable, reliable and authoritative. Kraepelin aimed to establish natural science causality and the laboratory experiment (as principal research paradigm). In the first edition of Kraepelin's textbook, he describes his position of 'psychophysical parallelism'. Kraepelin was firmly opposed to a position of 'spiritualism' or 'dualism', i.e. of a mental life (Seelenleben) independent of bodily function. He considerd pathological anatomy and clinical description the principal sources to identify correlates. Measuring 'elementary psychological functions', namely sensory perceptions (‚Apperzeptionen'according to Wundt) might help clarify the juncture between physical and mental phenomena. Kraepelin thought that psychopathological phenomena could be deconstructed ending up with elementary psychopathological 'particles'. Work on the effects of alcohol helped Kraepelin test this concept. Constructs into which he attempted to break down clinical alcohol effects were 'the act of choosing' (shortened) and 'discriminatory capacity' (prolonged/reduced). Postulating 'disease entities' was part of his experimental model. He thought of a science of 'the mechanics of mental disorders' in which mutual inferences were possible between specific causes and specific clinical manifestations. Epidemiology was another research method he considered important. In practical terms, his nosology was intended to provide guidelines for therapy, and also for the prevention and prognosis of individual cases of disease. Finding avenues to prevention was a distant aim. During the period in which Kraepelin built his nosological system, the 'basic sciences' were considered increasingly important for both the state and national economy, and substantial financial resources were devoted to research. In 1901, psychiatry became a compulsory subject in undergraduate medical training. Kraepelin's positioning of psychiatry amidst the natural sciences (rather than culture or education) strengthened its position in academia.

There was a parallel debate on 'degeneration theory' and 'population politics'. So-called 'hereditary biology mapping' (erbbiologische Bestandsaufnahme) of populatons was considered a priority task. This, according to Roelcke, was in line with Kraepelin's earlier position (in the 1880s) of the existence of negative effects of public welfare schemes on population health. Psychiatric epidemiology work and the study of mechanisms of heredity were to be two more pillars of a psychiatric science (and profession) that aimed at strengthening the 'Volkskörper' (whole population envisaged as 'body politic'). This was the beginning of the Genealogisch-Demographische Abteilung (Demographic Genetics Section) at the DFA founded in 1917, and Kraepelin's collaborator Ernst Rüdin was to lead that section. Rüdin's work was characterised by close cross-talk of science and politics, and, during the Nazi era, he was going to be important in planning and implementing the forced sterilisation programme and involved in the extermination programmes for people with mental illness, e.g. the T4 programme (Roelcke, 2000, 2012).

Kraepelin's late writings on nosology

In their detailed analysis of a late paper by Emil Kraepelin, Heckers et al. (2022) state that Kraepelin shaped the nature of our psychiatric diagnostic system. The authors analyse his paper 'Manifestations of insanity' that was a late synthesis of his approach in the light of the scientific debate with other authors (Hoff, 1988). Heckers et al. (2022) conclude that the text offers a conclusive description of Kraepelin's position and expresses his emerging interest in life history and culture. Kraepelin stuck to the concept of distinct disease entities but conceded that distinctions between them were 'often obscured by personality, life experiences, and/or cultural effects'. The authors describe Kraepelin's use of metaphors (human mind/brain as a machine with gears, the brain developing in stages, resulting in layers, and illness manifestations arising

in ways similar to the register of an organ producing sound), and they focus on the latter metaphor of the 'organ register'. According to Kraepelin, psychiatric disorders were related to three registers with distinct clinical features and brain-based mechanisms (first register – delirious, paranoid, affective, hysterical and instinctive forms; second register – schizophrenic form plus auditory-hallucinatory forms; third register – encephalopathic, oligophrenic, and paroxysmal forms). The authors refer to Birnbaum's impact on Kraepelin. Birnbaum considered pathoplastic factors that they described as 'innate peculiarities' and 'acquired conditions' shaping the patient's clinical picture. Kraepelin acknowledged that personality and life history shaped the appearance of mental illness. However, in offering his critique of Birnbaum's position Kraepelin also stated that any influence of pathoplastic factors was constrained by (and thus secondary to) underlying biology resulting from pathogenic factors.

Comparative psychiatry

Engstrom and Crozier (2018) refer to clinical research conducted by Emil Kraepelin at psychiatric hospitals in Java (1904) and in the USA (1925). Kraepelin thought that such observations could be useful in better understanding the relationship between ethnicity [Volksart] and mental disorder, and he intended to promote the field of 'comparative psychiatry'. Scientifically, both trips did not lead to findings helping Kraepelin to understand progressive paralysis (and whom among people with syphilis it afflicted) or the mental health effects of alcohol. Engstrom & Crozier see Kraepelin's work during both research trips as reflecting a 'larger neo-Lamarckian project of clinical epidemiology' and 'a reflection of his own long-standing diagnostic practices and research agendas'. In particular, Kraepelin thought (inconclusively) that findings of transcultural difference might assuage or confirm his fear of trans-generational degeneration in European populations. 'Weakened immune systems' in non-indigenous populations were discussed (GPI being rare among the indigenous Javanese people examined). The authors dispute that Kraepelin's work can be considered the outset of 'transcultural psychiatry'. They emphasise his efforts to expand the range of clinical data using diverse sources. In this endeavour, Kraepelin turned to local and regional asylums, juvenile detention facilities, Bavarian schools, military recruitment offices, and to the courts. The focus of his Java and USA research work of 1904 and 1925 revolved around questions on GPI and psychiatric sequels of alcoholism. Kraepelin's research on comparative psychiatry, according to Engstrom and Crozier (2018), attempted to consider social dimensions of mental illness. There was no focus on the concept of 'race'.

German Research Institute of Psychiatry (Deutsche Forschungsanstalt für Psychiatrie, DFA)

In reporting on the preparation for and foundation of the German Research Insitute of Psychiatry (DFA), Engstrom et al. (2016) describe the post-World War I (WWI) background. Some of the clinical work at DFA was related to soldiers who had suffered brain damage, and there was interest in what was referred to as 'pension neurosis' (Rentenneurose). The concepts of 'racial hygiene' and of 'degeneration' played a role. This, according to Engstrom et al. (2016), strengthened the genealogic-demographic section of the DFA (led by Ernst Rüdin). Kraepelin's endeavours in securing funding from philanthropists in the USA were accompanied by appeals expressed in an article that the DFA was essential to the survival of the German 'Volk' (people). Due to hyperinflation in 1923 Kraepelin was forced to seek state support for the DFA by joining the Kaiser-Wilhelm-Gesellschaft zur Förderung der Wissenschaften (KWG). Industry magnates such as Gustav Krupp and Carl Duisburg were involved in this move which secured financial survival of the DFA. KWG institutes drew on authoritarian and patriarchal models of industrial conglomerates, as well as academic models envisioning research facilities as built around an autarchic director. Engstrom et al. (2016) write that 'the DFA's research agenda embodied a complex web of scientific, economic, statistcal and bio-political agendas'.

Within the DFÀs psychology section there was an interest in Taylorism which was compatible with Kraepelin's long-standing interest in experimental psychology. The industrial psychology work recceived supported from large companies and Bavarian state agencies. Public health (Volksgesundheit) was a focus. Clearly, the DFA reflected Kraepelin's clinical research inerest and socio-hygienic concerns. There was a clinical work focus (distinguishing the DFA from Oskar Vogt's Brain Research Institute in Berlin). The impacts of alcoholism and syphilis were priorities. Kraepelin was involved in the temperance movement, and he repeatedly gave public lectures on the dangers of alcohol. Kraepelin's political views may have hampered the success of his quest for research funding during Kraepelin's trip to America in 1925. Adolf Meyer was outspoken in his correspondence albeit positive with

regard to support of the DFA by the Rockefeller foundation: '… the fact that Kraepelin himself represents a type of psychiatry which I hope will soon belong to the past and that he himself behaved rather disgustingly and will no doubt use the success in a not altogether pleasant way [that?] had best be put aside, because, after all, there will be others coming after him and there were meritorious workers with him at present'. (Engstrom et al., 2016, p 145).

Emil Kraepelin, an icon of psychiatric research

Engstrom and Kendler (2015) reiterate that Kraepelin, in the last third of the 20th century, became an icon of post-psychoanalytic medical-model psychiatry in the USA. The authors, in their paper, state that Kraepelin was more psychologically inclined, less brain-centric and nosologically less doctrinaire than is generally thought. The authors refer to Kraepelin's inaugural lecture at the Univeristy of Dorpat/Tartu (now Estonia) highlighting the influence of the methodology of early experimental psychologist Wilhelm Wundt (1832–1920). Kraepelin insisted on the importance of psychopathology in organising psychiatry as a clinical discipline, and he was sensitive to the role of psychological factors in the etiology of psychiatric illness (Engstrom & Kendler, 2015, p 1192). Kraepelin's somatic orientation was far from merely brain focused, but adopted a larger, systemic, whole-body approach including metabolic, serological, endocrinological, and other potentially 'auto-toxic' etiologic factors. In turning to his ground-breaking nosological work, Kraepelin stated that it was necessary to 'abandon for all time a systematic demarcation of mental disorders along the lines of Linnaeus' (Engstrom & Kendler, 2015, p 1193). Although he never doubted the existence of natural disease entities, Kraepelin was skeptical about whether he had in fact delineated such natural entities. The authors note that Kraepelin's critique of his own nosology grew over time.

Ideology and politics: threats to science

Michael Shepherd (Shepherd, 1995a) discusses Kraepelin's contributions to psychiatry, his research methods and results. The author cites Zilboorg (1941) saying that '… (the) history of German psychiatry of the nineteenth century is the history of psychiatric somatisation … In the middle of the century German psychiatry asserted the supremacy of the brain over any other structure and proceeded systematically to produce a psychiatry without a psychology' (Shepherd, 1995a, p 175). Shepherd describes the substantial contribution of Kahlbaum in the making of Kraepelin's classification, and he points out that studying large numbers of patients Kraepelin concentrated on the patterns of symptoms (over time) construed as biological facts rather than the mechanisms of disease. Subjective experience was minimised, and in the 6th (1899) edition manic-depressive psychosis was first presented as an entity distinct from dementia praecox. Kraepelin recorded clinical and demographic information systematically on index-cards (counting cards) treating the data numerically and subjecting them to personal interpretation. Shepherd highlights methodological shortcomings in Kraepelin's work and his ignoring the problems associated with sampling, observer bias and statistical analysis. Shepherd goes on to name the opponents and critics (Hoche, Bumke, Adolf Meyer, and Crow later on in time), and he concentrates on the clinical picture being more important than longitudinal course. He quotes work by Jablensky et al. (1993) describing Kraepelin's apprach as 'a natural scientits's approch of an astonishing breadth' (including psychologial and pharmacological experiment, statistics and technological innovation) with a view to continuous reformulation. Shepherd describes 'the man and his outlook', and he finds the traditions of German romanticism important in Kraepelin. Shephed points out a number of traits (in the 'private man'): religious interest, statements of his 'love of the fatherland' (which the author sets in the context of the rise of Bismarck's Germany to military, economic and political power). Shepherd describes Kraepelin as shaping German psychiatry and contributing to its rise in status and influence. Notions of superiority played a significant role; racial hygiene and theories of degeneration were referred to, and abortion, serilisation, selective breeding and the prevention of alcolol consumption were discussed (this applied to other countries, too).

Shepherd describes how Kraepelin was prepared to discard the objectivity of judgement in pursuit of patriotic ideas (see his medical and political work in the public domain, his views on the 'Volkskörper' or body politic, and on 'crime as a social disease' based on 'congenitally inferior predisposition'). Kraepelin considered mental hygiene an important field. Kraepelin's nationalism became more intense during WWI. There was a strong authoritarian element, and there were right-wing sympathies. Shepherd describes Kraepelin as assuming the role of a psycho-hygienic 'Führer', applying biologically based medical expertise to the political and social problems of the day. There were clear antisemitic statements, and Shepherd finds 'disturbing political overtones of proto-fascism'. He refers to Ernst Rüdin, Kraepelin's close associate and

psychiatric geneticist who joined the NSDAP and was involved in the racial hygiene programme that was to lead to a policy of genocide and the horror of the gas chambers. Shepherd responds quoting Jaspers and his critique of implicit philosophical concepts in Kraepelin, cloudy notions (of the 'will') and an idealistic version of the mind-brain nexus.

In a second paper, Shepherd (1995b) describes the social context of late 19th century Germany: Bismarck's chancellorship, the growth of national power, and a high value given to the natural sciences. In this context, the foundations of institutional and academic psychiatry were laid by one generation of psychiatrists. Referring to Kraepelin's use of counting cards Shepherd emphasises that there was a strong subjective element in his diagnostic and classification practice. He acknowledges later work that has shown Kraepelin's diagnostic dichotomy quite stable over time (and Kraepelin's acknowledging the role of personality). However, to document the subjective and ideological elements in his work, Shepherd (1995b) quotes Kraepelin (p 191) in dealing with mental health sequels of injury/trauma as

> '… more or less shocking phenomena of everyday life in which the mental constitution of the members of the community is manifested'.

Examples, according to Kraepelin, included

> '… suicide, crime, vagrancy and prostitution, the frequency and the motives for marriage, the tendency to produce and rear offspring, and the results of education in elementary and higher schools'; and '… to some extent military fitness, certain manifestations of political and religious life, migration from rural areas into towns, business enterprise, and much else …'.

Kraepelin continues:

> '… A mass psychiatry, having at its disposal statistics in their widest scope, must provide the foundations of a science of public mental health – a preventive psychological medicine for combating all those mischiefs that we group under the heading of mental degeneracy.'

Against the background of such ideological overtones, Shepherd highlights Kraepelin's reflections on mental health and society with reference to what he called the body politic (‚Vokskörper') and to his tenet of criminal behaviour being due to 'genetically inferior predisposition'. Kraepelin's attitudes towards alcohol and venereal disease provide further evidence of ideology and degeneration theory. During WWI Kraepelin took a clear chauvinist turn aiming to promote German 'greatness'. Kraepelin's endorsement of the concept of 'degeneration' across generations is obvious (with poverty considered a sequel and not a cause of 'inferiority'). Mental health physicians were to serve the military and economic strength of the nation. Social Darwinism was clearly there. Shepherd uses the term of 'proto-fascism'.

Debate

In a letter to the editor prompted by the paper on Kraepelin as an icon of psychiatric research (Engstrom & Kendler, 2015), Strous et al. (2016) criticise that, in referring to Kraepelin as an icon, Engstrom and Kendler had failed to refer to a number of other facets: Kraepelin's being a proponent of eugenics and 'degeneration theory', the anti-semitic statements in his writings, positions of nationalism and militarism in his writings, a right-wing socio-political agenda and his thinking on policies of 'racial hygiene'. In their rebuttal, Kendler et al. (2016) point out that their article did not intend to address the issues referred to by Strous et al. (2016) and that it is difficult to draw connecting lines between Kraepelin and later acts of his students and co-workers, and that, in their paper, they intended to cover other aspects of Kraepelin's work (while they acknowledge the importance of the topics addressed by Strous et al. (2016).

Discussion (part 1)

Emil Kraepelin was a seminal innovator in psychiatric research and nosology during his long tenure at the universities of Dorpat (Tartu), Heidelberg and Munich and at the DFA. His influence on international psychiatric classification and on the strategies guiding psychiatric research has survived for more than a century (Engstrom & Weber, 2007). His research strategy focused on severe mental illness but encompassed the whole spectrum of ill health including so-called minor (or common) mental disorders. There is a strong consensus that Kraepelin's programme was innovative including both clinical observation and psychology, serology, pharmacology, anatomy, and epidemiology. Kraepelin was ahead of his time in supporting epidemiology research, he persisted in pursuing the strategy of longitudinal clinical observation introduced by authors such as Kahlbaum and Hecker, and he formulated public mental health concepts. Kraepelin was highly effective in contributing to the establishment of psychiatry as a discipline. He helped to shape

psychiatry as a powerful societal institution the expertise of which was heard by German society and organisations of the state, and he contributed significantly to establishing psychiatry as an essential part of undergraduate medical training and clinical medicine (Hoff, 2015).

However, Kraepelin did so from what was a nationalistic, at times chauvinistic and ideological perspective. As many (but not all) psychiatrists of his time he made use of the scientifically vague concept of 'degeneration'. However, he refrained from overgeneralization by suggesting significant differences between diagnostic groups with respect to the influence of 'degeneration' (Hoff, 2008). In general, there was insufficient critical methodological focus in some of his research, and pre-conceived concepts determined much of his science (narrowing its methodological power). His subordinating psychiatry to societal power, to nationalism, chauvinism and other extraneous ideologies must be borne in mind.

There is considerable consensus, in the literature, that Kraepelin, in his research, was influenced by a number of a priori views, i.e. predetermined views or prejudices without addressing them in his writings (Weber & Engstrom, 1997). These included:

- Kraepelin's tenet in organising his clinical research around the construct of 'natural disease entities' ;
- Kraepelin's assumptions regarding the construct of 'mental disorder' (psychophysical parallelism, positivist research agenda, neo-Lamarckian project of clinical epidemiology); and
- A number of views on society (working in the interest of a 'Volkskörper' or 'body politic', nationalist-Chauvinist positions, German 'greatness', and societal 'degeneration' across generations as a target for population politics and public health measures).

There is a serious argument that Kraepelin, toward the end of his career, moved in the direction of a blurring of nosological borders, and that he opened up to the influence of life history and culture in the reality of mental illness over time. His use of the term of 'organ registers' (that resonates with some of the concepts of part 2 below) can be considered as underlining the multiplicity of symptom combinations, and his late writing indicates some blurring of lines between illness constructs while he did hold on to the construct of natural disease entities. In Kraepelin's time, there was a lively scientific debate and controversy around his work with a number of authors formulating substantiated criticism (as taken note of in the pertinent literature).

In summary, there is concern that Kraepelin's research strategy was clouded by the strength of a number of preconceived views not made explicit, by his narrowing down the field of enquiry and by the impact of non-scientific, ideological positions on his research. There may have been an over-emphasis on observable behaviour with insufficient attention given to individual experience. The nosological building that Kraepelin erected has been the most influential one across psychiatry. This has been the case because his conception has helped researchers organise both the multitude of clinical findings and the heterogeneity of research findings. However, there is reason to believe that Kraepelin's work in its entirety has failed to encompass the field of mental ill health in its full (experiential, cultural and scientific) width.

Part 2: Walter Benjamin's working methods

Materials and methods

Walter Benjamin's approach to other authors' work, his use of a range of materials and texts and recurring themes of his work are described using a number of books and articles. Seven texts were chosen to illustrate different aspects of Benjamin's work. These include (multi-author) work on the materials and methods he used, the theories referred to and elaborated in his texts, the methods of enquiry as descibed by Benjamin and by other authors. Books on Benjamin's philosophical positions and on the importance of visual perception in his work are considered. Benjamin's methodological contribution to the fields of international relations and international political sociology is discussed. The selection is not systematic, it is owed to the attempt of describing Benjamin's writings in a way that might help understand his working methods and provide indications of his overall work plan and intellectual project. This contribution adds to articles introducing mental health professionals to Benjamin's work and his personal and intellectual formation (Ikkos et al., this volume; Ikkos & Stanghellini, in press).

Results

Theory

A book by Kramer (2004) deals with Benjamin's overall theoretical work. The author describes Benjamin's work with a focus on four themes: theory of language, aesthetic theory, media theory, and the theory of

history. Benjamin, according to Kramer, develops a set of concepts relevant to art criticism. There is an emphasis on language with a view to physical, sensual and material aspects. Benjamin's thinking suggests that language 'produces reality'. His aesthetic theory refers to allegory and to Leibniz' concept of the monad (with details containing and expressing the whole). Benjamin highlights the influence of technological change on individual works of art, on our look at art, and on art production. Kramer describes the exchange character of commodities (i.e. what Benjamin, using a term coined by Marx, refers to as the commodity fetish) in modern capitalist societies and its role in Baudelaire's poetry as central to Benjamin's thinking. In the 1930s Benjamin was important in developing the literary format of the essay as a new way of writing criticism. With reference to history, Benjamin developed a specific concept regarding the relationship of past and present with aspects of a past world dialectically 'exploding' into the present helping us to better understand and, perhaps, opening a space for hope in shaping the world. In this context, Benjamin defines the 'dialectical image' as a textual phenomenon. With respect to psychiatry, there is little explicit cross-talk in Benjamin's writing. However, stepping back, Benjamin's work could be considered a systematic approach to human 'experience', to our 'perceiving the world' and to proceeding from the sensual to the conceptual. Benjamin's work concentrates on how the external world relates with experience, and on how the past and present interact.

Micrology

A text by Meiffert (2017) deals with the concept of 'micrology' in Benjamin's work. Meiffert states that the relationship between the phenomena we perceive and the world of ideas is central to Benjamin's thinking. The empirical world, through images, dissolves in the world of ideas. Concepts work on the phenomena to arrive at the 'Gestalt' of ideas. The world of ideas relates to things as constellations (Sternbilder) relate to individual stars. Individual items of experience (Erlebnisse, related to phenomena) form into and inter-connect constituting personal experience (Erfahrung). Benjamin's understanding posits a moment of (potential) recognition of the past in any one moment of present time (now-time, Jetztzeit) with recognition being tied to the discovery of 'resonance, similitude or sense' in the encounter of present and past. There is a moment of recognisability in his theory of the relationship between past and present. This part of Benjamin's thinking is relevant in the late text of his 'Theses on history'. Benjamin's concept of resonance in the encounter of present and past leading to moments of recognition or 'dialectical images', with the past exploding into now-time, could be useful in the attempt to understand therapeutic encounter. Also, the concept of 'remembrance' (einsichtsvolles Erinnern) could be relevant in mental health contexts. Mental health theory and practice could take on Benjamin's action of deciphering individual experience and of embedding individual experience into the wider cultural context (Ikkos, 2024). We could hypothesise that Benjamin developed a language to describe the concept of therapeutic encounter and 'moments of discovery' in psychotherapeutic relationships.

Figures of thought

Leifeld (2000), in a monograph, deals with Walter Benjamin's concept of 'figures of thought' (Denkbilder) (Eiland & Jennings, 2016). Benjamin's 'figure of thought' texts refer to personal memories, to the personal detail of family life, and some refer to detailed descriptions of wider societal life. Haptic and sensory experience is important in many of his figures of thought. Leifeld describes Benjamin's figures of thought as 'thoughts in the making'. Leifeld also refers to a kind of 'negative theology' and Jewish mysticism as points of reference for Benjamin, and she identifies an element of hope towards emancipation and democratisation. Benjamin's figures of thought could be read as ways of capturing subjective perceptions of the world, and they could be considered a kind of propaedeutics to understanding individual history (or prepare us in the art of history-taking in psychiatric practice) (Stanghellini & Ikkos, 2024).

Dialectics of seeing (Arcades Project, Passagen-Werk)

Buck-Morss (1991) presents a fascinating reconstructive re-reading of Benjamin's (unfinished) Arcades Project. The themes of her book include a focus on city life and urbanism (a fleeting modern mythology), Benjamin's political concern, and his interest in the philosophy of history as over-riding targets of his opus magnum. Benjamin intended to use the products of mass culture in his Arcades Project. He developed an elaborate filing system (Konvolute). The Arcades Project, according to Buck-Morss, should be read as a work of history of philosophy, its author being committed to a graphic representation of truth with historical images visualising ideas. Benjamin's

approach was to use montage in assembling the material (quotes, excerpts), and he was convinced that theory would spring out of the material rather than having to be inserted in it. There is an emphasis on rapid technological change as a major driver of history, and Buck-Morss emphasises the montage element (as part of Benjamin's method), and technology and art are considered to inter-relate. Commodity worship in a capitalist world is thought to be close to modern 'dream worlds', and Baudelaire's poetry is considered to express the experience of self-estranged humanity in 19th century urban settings. There is a marked interest in utopian oncepts (Fourier, Saint-Simon) in the Arcades Project. Buck-Morss (1991)considers technology as central to the book, with architecture, engineering, painting and photography, literature and mass journalism considered a tangle of contradictory elements centered around consumer dreams and commodity worship. There was also Benjamin's belief in cultural history being central to (working) class education. Historically, Benjamin's thought was of a 'shock' of recognition leading to the juxtaposition of past and present (a 'dialectic of awakening' pointing to now-time and immediate future). In sum, Buck-Morss (1991)presents a close reading of the Arcades Project as a superb cultural history of modern capitalist society with a view to bringing about political change.

Material and concepts

In the preface of a multi-author book discussing various aspects of Benjamin's style of working and theoretical background (Voigt et al., 2019), the editors state a lack, in research on Walter Benjamin, of critical historical attention, i.e. a lack of research focusing on the historical context of the materials which Walter Benjamin used in his work. The book is dedicated to Benjamin and the sources, materials and fellow authors that he referred to in his works. It is a compilation of intellectual/scientific cross-talk and references that were relevant in Benjamin's writing. There are repeated references to fellow thinkers such as Carl Schmitt (focusing on the souvereign and self-referentiality of power) and Neo-Kantian philosoper Hermann Cohen. There is reference to Benjamin's struggle to find a format for his text on a stay in Moscow that would reflect the complexity of his impressions. There is repeated reference, in the essays of this volume, to Benjamin's focus on 'material substance' in language and other materials depicted in his work, and the various texts emphasise Benjamin's focus and reliance on quotation and montage. Benjamin's montage method is distinguished from Bertolt Brecht's use of the so-called 'V effect' (alienating, or Verfremdungseffekt) and the use of quasi-experimental arrangements in his epic theatre. Blanqui's utopian work is referred to emphasising the role of utopian thought and how that had an impact on Benjamin. It is clear that Benjamin reflected a wide range of materials (e.g. texts by cultural critic Eduard Fuchs) and that he reflected historic-materialist theory of nature and of history (and of the relationship between economic infrastructure and cultural superstructure with considerable autonomy ascribed to the latter). With respect to the mental health field, the emphasis here is on the breadth of Benjamin's intellectual search and his meticulous pursuing of different (and often conflicting) lines of thought and political theory. His intellectual search was certainly broad, methodological and non-ideological, and overall societal change was a perspective underlying his opus magnum.

Philosophical concepts

Friedlander (2012), in a book on philosophical aspects in Benjamin's work, considers the limitations encountered by any attempt at identifying the author's philosophical position. These limitations are due to Benjamin's practice as an art critic and to his preference for the essay (as a literary format). Also, the aphoristic and fragmentary nature of many of his works makes it difficult to identify his philosophical thinking. In the final chapter on 'Eingedenken' (or remembrance) Friedlander concentrates on Benjamin's approach of seeking a 'messianic arrest' of history with the dreams of the past realised in a new form, i.e. as a force driving the present forward. The reference to messianic thinking is not overtly religious. The multitude of materials of the past, in Benjamin's understanding, could be transformed into a call to action for the present. In terms of mental health considerations, we notice a form of optimism based on discovering the past as it unfolds into an understanding of the present (which includes the dimension of hope).

The colour of experience

Caygill (1998), in a book entitled 'Walter Benjamin. The colour of experience', reads Benjamin's work as pursuing different interpretative strategies and comprising a wide range of formats of literary work (ranging from short critical articles through academic metacritique to texts of philosophical criticism). Caygill describes Benjamin's search as an attempt at

extending the limits of experience as treated in philosophy including the worlds of art, religion, language and urban life. Benjamin's text on the 'Work of Art in the Epoch of its Technical Reproducibility' is in this line of work. At the heart of Benjamin's philosophy, according to Caygill, is a radical transformation of the concept of experience (departing from Kant's concept). He attempted to break down the distinctions between intuition, understanding and reason. His thesis on 'The Origin of the German Mourning Play' bears the mark of his interest in the socio-political origins and consequences of modernity. He found the concept of allegory useful in clarifying the relationship between appearance and essence. Benjamin dealt with photograpy and film as art forms, and he reflected on the re-creation of 'aura' in photography. He was interested in concepts of experience responding to changes in technology, and he saw changes in the quality of experience shaped by technological progress. He was open to all aspects of urban living. City life, fetish commodity, and urban marginal figures, according to Benjamin, combine to build the urban environment. In Baudelaire's work that was central to the Arcades Project, the 'blosssoms' of progress are blighted by remembrance of their transience. Benjamin's Baudelaire studies identified the lived experience of 'shock' as the principle informing Baudelaire's poetry. The Arcades Project was to deal with all cateorgories of urban experience. Caygill (1998) presents the evidence suggesting that Walter Benjamin's critical writing is, in essence, visual- and colour-oriented and that his speculative theory of experience rests very much on a visual experiential basis.

Benjamin, international relations and political sociology

In a text by Pusca (2009), the relevance of Walter Benjamin's work to the fields of international relations (IR) and international political sociology (IPS) is described. The paper refers to the so-called aesthetic turn and surging interest in everyday life and popular culture within the social sciences (and IR and IPS, in particular). Benjamin's work has been widely referred to in the fields of IR and IPS. Pusca argues that Benjamin's practice of studying change through cultural transformations and his transcending the limitations of text through the use of images and textual collage are both important. The author concentrates on Benjamin's interest in the material or visual rather than classical text as a central element in the search for knowledge about society, culture and 'now-time'. Pusca (2009) highlights Benjamin's interest in collecting varied materials (Marx et al., 2007), in individuals and groups marginal to social systems and in the dialectical relationship between (recent) past and present. She refers to Benjamin's preference for the scientist immersing himself or herself into daily life and into the rhythm of the city as a method of understanding, and she considers his method of study as holding significant promise for the field of IR and related fields of social science.

Discussion (part 2)

In a nutshell, and referring to Benjamin's wide-ranging literary, cultural, aesthetic and historical research and writing, we could tentatively formulate as follows:

1. Benjamin's work could be considered a systematic, literary and cultural science approach to experience.
2. Benjamin's concept of resonance in encounters of present and past leading to moments of recognition or dialectical images, with the past exploding into now-time, could be useful in understanding therapeutic encounters.
3. Benjamin's figures of thought could be read as ways of capturing perceptions of the world, and they could be considered a kind of propaedeutics of understanding individual history and experience.
4. The Arcades Project (Passagen-Werk) can be considered a superb cultural history of early 19th century capitalist society with a view to bringing about political change in the 20th century.
5. With respect to materials and methods used by Benjamin, we take note of the breadth of Benjamin's intellectual search and his meticulous pursuing of different (and often conflicting) lines of thought and political theory.
6. Benjamin's understanding of the past opening into now-time could pave the way for hope (in the form of glimpses of potential action) based on discovering the past as it unfolds into an understanding of the present. This might help in understanding subjective change as people grapple with difficulty in marginal social situations.
7. There is substantial evidence suggesting that Benjamin's critical witing and philosophy is, in essence, visual- and colour-oriented and that his speculative theory of experience rests very much on a visual experiential basis.

8. A wide range of (national and international) societal phenomena can be studied using the methodology proposed by Benjamin, a method prioritising the visual over text and marginal subjects (and also objects) over the normative societal panorama.

General discussion

This text attempts to describe and understand the thinking of two outstanding scientists. Walter Benjamin died at a young age which prevented him from fully applying the method of inquiry he had developed. Benjamin outlined his methodology or way of proceeding as a highly elaborate form of 'literary montage' against the background of an all-encompassing knowledge of literary, cultural and social science theory. He had successfully presented and defended his PhD (in literary science). The young Johann Wolfgang Goethe-University in Frankfurt was very close to rejecting his 'habilitation thesis' (which he withdrew when rejection appeared almost certain). This meant that, formally, he was not in the position of independently teaching German university students. It can be hypothesised that he was unable to take that step because the reviewer was unable to grasp the content of his habilitation thesis (on the German Mourning Play of the Baroque era). His success as a literary critic in Weimar Germany was impressive, and his effect on literary, cultural (and political) science since 1945 has been phenomenal. His work can be considered a literary and cultural scientist's work (with an interest in utopian thinking, Jewish mysticism and Karl Marx' writings). Thus, there is a marked distinction between Kraepelin and Benjamin, the latter being marginal, in almost permanent financial difficulty and on an (ambivalent) road to exile and destitution.

Emil Kraepelin, on the other hand, succeeded three times in being offered a chair in psychiatry (and accepted all three consecutively), and he managed to found and build the German Research Institute of Psychiatry (DFA). He also became a dominant psychiatric scientist and science strategist of his time. He profoundly influenced psychiatry as a medical discipline in the making, he significantly contributed to the establishment of psychiatry as a discipline associated with institutions of societal and medical power, and he did not hesitate, in some of his statements, to blur the lines between convictions, ideologies and scientific fact. His effect on world psychiatry in both conceptualising the nosology of mental disorders and organising high-quality research on the etiology and course of mental disorders has been impressive, with senior experts of the field continuing to refer to the milestones of his thinking.

The importance of material culture, in both Kraepelin's and Benjamin's work, might help in bringing them together. The counting cards used by Kraepelin, and ample citation, visual data (photographs, film, etc.) and convolutes of excerpts used by Benjamin bear some resemblance in drawing conclusions from material records. Both authors knew the advantages of systematisation. Kraepelin's metaphor of the 'organ register' could be another point to bring them together. Heckers et al., 2022) describe Kraepelin's use of metaphors (human mind or brain as a machine with gears, the brain developing in stages, resulting in layers, and illness manifestations arising in ways similar to the production of sound by an organ register), and this reminds us of Benjamin's use of allegory and montage. However, the metaphor of the organ register, to Kraepelin, was about distinct clinical features and brain-based mechanisms. Benjamin's approach was dependent on the material culture of the day (or recent past), with a rich cultural heritage referred to. His focus included modern materials such as glass and steel. In sum, both the reference to material reality and the use of collecting activity made by Kraepelin and Benjamin suggest little similarity beyond the recognition of the importance of the world's material base.

Can anything be taken from a comparative look at the distant methodologies and contributions of these outstanding figures? What springs to mind is the distance between the two rather than any cross-talk of their writings (as though they had been living in different worlds). The obvious argument is that Kraepelin and Benjamin operated in distinct and non-related fields of enquiry, that they were distant in terms of discipline (medicine in Kraepelin vs. literary and cultural criticism in Benjamin) and cultural orientation (marked philosophical realism and positivism in Kraepelin, and a background in post-Kantian thinking with an interest in Jewish mysticism and Marx in Benjamin). However, let us explore whether any cross-talk is possible and holds potential. We do that by considering

- a functional, (psycho)motor, cognitive (and cognitive-affective) perspective of psychiatric research and
- the relevance of non-professional (or peer or service user) contributions to the understanding of and coping with mental illness.

In continuing the overall project of inquiry on which Emil Kraepelin had embarked, the study of brain function is indispensable. Kahn et al. (2015) discuss schizophrenia as a cognitive and behavioural disorder, and they argue that it is related to how the brain processes information. Neuroimaging studies have confirmed impaired information processing. Walther and Heckers (2024) refer to disturbed psychomotor behaviour and its cerebal control as relevant in psychotic and mood disorders, and they argue that recent findings regarding intereffector regions in the brain are relevant in organising functional and dysfunctional psychomotor behaviour. There is a scientific debate around how much of the phenomena of psychotic illness can be understood through impaired cognition (Heckers, 2013; Kahn & Keefe, 2013). Any observer seriously considering the problem would go along with the importance of different approaches (to cognition, affect regulation and psychomotor function) in the attempt to better understand schizophrenia or psychotic disorders. There is also the unsettled debate around distinctness vs. continuum in elucidating the broad spectrum of psychotic disorders.

What is it that springs to mind, in a mental health context, when we reflect on Walter Benjamin's work? Could we argue that Benjamin, in his multi-faceted writing, elaborated a model of human 'experience' in its relationship with the multitude of phenomena of the external world (Morgan, this volume)? We could argue that the perceptions in awake mental states and dreams (as productions, marks or writings rather than inner experiences) that Benjamin translated into literature provide a view of the world as seen in the first half of the 20th century. Benjamin argued that this configuration had been pre-figured by a number of overlapping 19th century 'worlds' and that an understanding of current events could be facilitated by 19th century constellations reaching into or pointing to the world of post-WWI Europe. The figures of thought arising from the recent 19th century past reflected by Benjamin could be described as dialectical images (around objects, material, architecture, and 'marginal urban worlds') revealing to us, as readers, the stable features of 20th century society such as the commodity character of experience, and the revolution of image production shaping human world views. In this way, Benjamin could be considered a key scientist (philosopher and collector) of the activities of 'modern mankind'.

Benjamin's work could be read as essential in the propaedeutics of mental health science and practice. He would be seen as having provided us with a cultural perspective on the wider (human and material) world that provides the background to all individual experience. He achieved this by weakening elements of logic and causal chains of events over time, and by blurring the line between cognitive appraisal and immersion (in experience). He was open to identifying the allegorical in political or cultural phenomena. What Benjamin provides us with is an apperception of human experience that springs from the recent past, may help us understand the present and opens vistas for a way forward. Psychiatric science and teaching, in the tradition of clinical research, have the task to collect and analyse a wide range of data on mental illness. Both Benjamin and Kraepelin were looking for a comprehensive perspective of 'recognising' with Kraepelin, more narrowly, seeking the quantification of mental or 'subjective' phenomena, and with Benjamin aiming at an all-encompassing experiential representation.

However, there is an argument of keeping the two perspectives apart as Benjamin's thinking does not address mental ill health (as Kraepelin did). In this line of thought, Benjamin's contribution to psychiatry could lie in mental health propaedeutics and a basic science of experience (Morgan, this volume). His concepts extend beyond the world of mental ill health and suffering. There is a potential, in Benjamin's approach, of exploring experience in its relationships with history, culture, and the socio-economic sphere to enrich psychiatric or mental health theory and practice. Having a theoretical apparatus to fully describe how individuals perceive the material world (and how that relates with the recent past and present circumstances) would help ensure that psychiatric practitioners strive to consider the whole gammut of human eperience. However, Benjamin's thinking, as opposed to Kraepelin's work, never set out to map and analyse mental ill health (in the strict sense).

Paul Hoff (2023), in a recent book on German-born Jewish psychiatrist Arthur Kronfeld (1886–1941), referred to Kronfeld's book 'Das Wesen der psychiatrischen Erkenntnis' (Kronfeld, 1920). Kronfeld argued that psychiatry of the early 20th century had uncritically accepted the dominance of research concepts of neighbouring (natural, biological) sciences, and of neurobiology, in particular. Kronfeld argued that it was for this reason that psychiatry failed to integrate the original phenomena of the psyche in an autonomous, systematic scientific theory. Kronfeld called for what he termed 'Autologie' (autology), i.e. psychiatry, in his view, ought to describe and understand the mental health field in all its variants and forms of pathology as an object of scientific enquiry in its own right, and he postulated that genuine psychological

methods of investigation had to be developed to that end. Hoff (2023) argues that Kronfeld's position reflected the programme of a Neo-Kantian foundation of psychiatry as an exact science avoiding the errors both of 'materialism' and of 'idealism'. Kant held that a priori categories (reine Verstandesbegiffe) were not applicable to the phenomena of mental health and mental ill health. In turning back to Walter Benjamin, we could argue that Benjamin, in his writings, developed an outstanding tool box or methodological apparatus of how psychic phenomena can be conceptualised in a dynamic and material perspective open to transience and history.

The field of psychiatry has, in recent decades, experienced a phenomenal and astounding enrichment and challenge. During the last twenty years there has been an enormous surge in research, practice models and debates around peer or service user perspectives in both psychiatric practice and mental health research (Åkerblom & Ness, 2023; Bellamy et al., 2017; Cabassa et al., 2017; Watson, 2019). Theoretical formulations of models of action of peer support in bringing about change have been published (Gillard et al., 2015). There is evidence that peer support models are effective in improving well-being, social functioning and recovery. The rise of this model is linked to the demand for unconditional participation in society and full respect of personal autonomy owed every person affected by mental illness. The current strength of this position (which is close to cultural hegemony) can be seen as an accomplishment of recent centuries accomplished through struggles fought in industrialised countries (and taken up in post-colonial debates). However, there has also been the argument that, in the peer involvement and peer support movement, there could be elements of a neo-liberal model of continuous self-improvement or self-reformulation as part of a neo-liberal Zeitgeist (Braslow, 2013; Matuschek et al., 2023).

The research field around mutual support, user involvement and peer support points to non-psychiatric conceptualisations of the issues that are at stake. Although peer-led research and peer support practice models have been implemented within psychosocial support services, such models and reseach activities can also focus on practical action and theoretical concepts reaching beyond the mental health field. This fact points to wider social, cultural and political issues, and we could take interest in neighbouring fields such as mad studies (Beresford & Rose, 2023) or subaltern studies (Zaib et al., 2015) which constitute related but distinct fields of inquiry. The field of action and of scientific inquiry relevant to mental ill health and suffering is wide, and psychiatry is only one stone in that wider puzzle (Stanghellini, 2022). Walter Benjamin's work may be helpful (to psychiatric thinking) in building bridges with respect to how the world is viewed and experienced by humans and how that relates to the strength, successes and failures of psychiatry. Emil Kraepelin was an eminent protagonist of the field in understanding the importance of observation, time course, outcomes and illness correlates in understanding the episodes and longitudinal course of mental disorders. His work can be used in guiding the efforts at further improving functional approaches to understanding pathogenesis. All these efforts, however, operate within a sub-system of the wider historical world (as a material and sensual ensemble). Walter Benjamin has paved a way of understanding that wider world. It could be valuable to explore his concepts and contrast them with the construct of diagnosis.

In conclusion, we could use the 'figure of thought' concept (Eiland & Jennings, 2016) as applied to Emil Kraepelin and Walter Benjamin in sorting out the ideological infringements on science that we have identified in Kraepelin's work and the limitations of speculative thinking in Benjamin's writing. That effort could open new perspectives for multi-faceted mental health research, and it could, as an approach to science, reach beyond psychiatry. We think it is unlikely that Kraepelin and Benjamin would have wanted to share in such a joint effort. However, their distinct and towering contributions can help us in formulating how to continue our search to better understand mental suffering, a search that is both scientific and cultural, in the end.

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